

SOCIO-DEMOGRAPHIC DETAILS

Name:	Praveen Shrivastva	Age:	50 years old
Gender:	Male	Date of birth:	06/06/1976
Education:	BA LLB	Religion:	Hindu
Socio-economic status:	Middle Socioeconomic Strata	OPD No.:	20250249254
Address:	Hanuman colony, Guna	Contact No.:	9425133717
Test administered on:	09/01/2026	Aadhar No.:	

Informants:

Name:	Relationship:	Duration
1. Ram Niwas Shrivastava	Father	Since birth
2. Shekhar Vashishth	Neighbour	Past 30 years

The information given is reliable and adequate.

CHIEF COMPALINTS:

- 1.) Repeated unusual and unrealistic beliefs
(e.g., “Main machhar ban kar udta hoon, Main Jawaharlal Nehru hu”)
- 2.) Keeping doors and windows closed, avoiding interaction
- 3.) Speech that is difficult to understand and does not match the situation
- 4.) Reduced emotional expression and inappropriate reactions
- 5.) Lack of motivation and reduced interest in work

8 years

Reason for Consultation: Patient came for pharmacological treatment and behavioral assessment.

HISTORY OF PRESENT ILLNESS:

Total Duration: Total Duration of illness is 8 years.

Onset: The onset of illness is insidious.

Course: The course of illness is chronic.

Progress: The progress of illness is deteriorating.

Precipitating Factor: The onset of the patient's current difficulties appears to be associated with the stress of failing a government examination, particularly in the context of peers achieving success, which may have significantly impacted his self-esteem and sense of competence. Additionally, certain aspects of his premorbid personality, such as high expectations from self, tendency to become easily overwhelmed under stress, and reduced coping ability, may have contributed to the emergence of these difficulties

Perpetuating Factor: The continuation and worsening of his condition appear to be influenced by multiple factors, including poor awareness of his condition, which limits help-seeking and treatment adherence. Furthermore, ongoing financial stress within the family and limited psychosocial support act as maintaining factors. These, combined with his reduced motivation and withdrawal from social and occupational roles, have contributed to the persistence and gradual deterioration of his overall functioning.

Brief History:

The Index patient, P.S., is a 50-year-old unmarried male, educated up to BA LLB, previously working as a lawyer but currently unemployed, residing in Guna with his father and brother. He was brought to the mental health institution on 9th January 2026, largely under pressure from family members. The patient was asymptomatic until 2018. The patient reported chief complaints of unusual and unrealistic beliefs, keeping doors and windows closed, reduced emotional expression and inappropriate reactions, lack of motivation and reduced interest in work from the past 8 years.

In the presence of unusual and unrealistic beliefs, he frequently says things such as "Main machhar ban gaya hoon," "Main Jawaharlal Nehru tha," and "1947 mein jhanda maine fahraaya tha." At times, he also says "Main bakra hoon, mujhe halal kiya jaata hai." These ideas are not fixed and tend to change frequently. His thoughts appear fragmented, and he finds it difficult to stay on one topic, often shifting from one idea to another without clear connection. His speech has become difficult to understand, with sentences that do not fit the context or situation. Thoughts are always preoccupied with religious, political and philosophical abstract themes.

Over the years, he has increasingly withdrawn from others. He prefers to stay alone in his room, keeps doors and windows closed, and avoids interacting with family members or others. Informants report that he spends a significant amount of time talking to himself and is often seen wandering aimlessly within the house. Changes in his emotional responses have also been observed. He shows very limited emotional expression and, at times, reacts in ways that do not match the situation. For example, during the death of his mother two years ago, he did not show any visible emotional reaction and did not participate in rituals. At other times, he becomes irritable or angry without any clear reason and experiences frequent changes in mood. The onset of these difficulties is reported to be around the time when he was unable to clear a government examination, while many of his peers were successful. Around that period, he had brief employment for about one year, during which early behavioral changes were noticed. Over time, his ability to work declined, and he eventually left his job. Currently, he remains unemployed and does not express any clear plans or goals for the future. He is often described as lacking motivation and energy.

His daily routine is significantly affected. He has reduced involvement in household or social activities and spends most of his time alone. Informants report that he has difficulty maintaining sleep for the past several years, although his appetite has remained largely adequate. Family members report limited social support and ongoing financial stress, which have added to the overall difficulties in managing his condition. The patient continues to deny having any problems and has limited understanding of his current situation. The patient's condition has shown a gradual worsening over the past eight years, affecting his thinking, behavior, emotional responses, social interaction, and occupational functioning.

BIOLOGICAL FUNCTIONING:

The patient's biological functioning shows noticeable disturbance in some areas. His sleep has been consistently poor for the past several years, particularly with difficulty maintaining sleep, as reported by the informants. Despite this, his appetite appears to be largely adequate, with no significant changes in food intake noted. His personal hygiene and self-care are reduced, as he tends to remain isolated indoors and requires prompting for daily activities. No significant concerns have been reported regarding bowel and bladder habits, which appear to be within normal limits. Information regarding libido is limited; however, given his overall reduced motivation and social withdrawal, it appears to be decreased. In terms of occupational functioning, the patient was previously employed as a lawyer but has been unemployed for several years. He left his job due to a gradual decline in his ability to function effectively. Currently, he does not engage in any productive work, lacks future planning, and shows minimal motivation to seek employment, indicating a significant impairment in occupational functioning.

NEGATIVE HISTORY:

1. No history suggestive of brain fever, head injury, vomiting and fits-like symptoms such as frothing of mouth, rigidity in body, rolling of eyes.
2. No history suggestive of intake of any substance, alcohol, weed, nicotine and caffeine.
3. No history suggestive of indulging into repetitive thinking and doing behavior.
4. No history suggestive of increased talk, self-boasting, high mood and increased energy, decreased need for sleep.
5. No history suggestive of crying spells, sad mood, lack of energy, loss of interest, ruthlessness, helplessness and rejected feelings.

TREATMENT HISTORY:

Medical History: The patient has no significant past medical history.

Psychiatric History: The patient reportedly consulted a psychiatrist in 2016, where a provisional diagnosis of depression was given. However, no treatment records are available. The duration of treatment was approximately two months, following which the patient discontinued consultation.

FAMILY HISTORY:

Family History of Medical/Psychiatric Disorders: There is a reported history of intellectual difficulties in the patient's brother; however, no formal diagnosis or records are available.

Living Arrangement: The patient currently resides with his father and brother.

Primary Caregiver: Father is the primary caregiver.

Type of Communication: Communication within the family is reported to be open and honest.

Family Understanding of Illness: The family has partial understanding of the patient's condition.

Family Members Attitude Towards the Patient: Family members are described as supportive and caring toward the patient.

Family Stressors and Discords: Significant financial stress and burden related to illness and disability are reported within the family.

PERSONAL HISTORY:

Birth and Early Development: The patient was born through normal vaginal delivery after full-term gestation. Birth cry was present, and no complications were reported. Developmental milestones were achieved on time, with no history of developmental delays.

Behavior During Childhood: During childhood, the patient exhibited temperamental difficulties, including frequent temper tantrums, crying spells during anger, and withdrawing behavior such as going to the terrace when upset.

Physical Illness During Childhood: History of nail biting was reported during childhood. No other significant physical illnesses were noted.

Home Atmosphere During Childhood and Adolescence: The home environment was reported to be conflictual, with frequent interpersonal disagreements.

Scholastic and Extracurricular Activities: The patient started schooling at the age of six years and was reported to be friendly in school. He completed his education up to BA LL.B. from a PG college. He was a good cricketer and actively participated in the sport.

Occupation: The patient worked as a lawyer in Guna and was employed around the age of 28 years. He is currently unemployed.

Sexual / Masturbation History: No information could be obtained, as the patient was not comfortable discussing this area.

Marital History: The patient is unmarried. As per history, he remained highly focused on his studies, which contributed to not pursuing marriage.

Substance Use History: No significant history of substance use reported.

Forensic History: No significant forensic history reported.

General Pattern of Living: At present, the patient wakes up around 8:00 AM and spends most of the day inactive. He does not engage in any structured activity, shows poor self-care, and wanders aimlessly within the house.

PREMORBID HISTORY:

The patient's premorbid personality suggests that he was previously ambitious, hardworking, and highly focused on academics and career. He was described as responsible in his work and maintained good standards of grooming and hygiene. Socially, he was popular and had multiple acquaintances; however, his relationships were not long-lasting, possibly due to his strong focus on studies and limited emotional investment. His reaction to stress was characterized by a tendency to become easily overwhelmed, with reduced motivation following stressful events. He was somewhat shy and preferred limited social interaction despite being socially capable. Interpersonally, he had difficulty adjusting to new situations. In terms of intellectual activities and hobbies, he showed interest in cricket and reading and was actively engaged in these pursuits. He also demonstrated a tendency toward daydreaming and imaginative thinking. His general energy levels were high, and he was enthusiastic about work and studies. Emotionally, he experienced periods of low mood and tended to remain withdrawn at times. His attitude toward self was positive, and he maintained neatness and cleanliness. He held moderate religious beliefs, along with high moral standards and expectations from himself.

MENTAL STATUS EXAMINATION

1. GENERAL APPEARANCE & BEHAVIOR:

Physical appearance: The patient appeared well-kempt and was dressed in neat, clean, and age-appropriate clothing.

Entered into the Room: The patient was brought to the interview room with coercion and initially showed reluctance to participate.

Facial expression and body language: The patient appeared uninterested and internally preoccupied. His facial expressions were reduced and did not consistently match the context of interaction.

Personal hygiene: Personal hygiene was adequately maintained. Nails were trimmed, and overall grooming was appropriate.

Eye contact: Eye contact was established but not maintained for a sustained period.

Attitude Towards the Examiner: The patient was cooperative during the interview.

Rapport: Rapport was established with some effort and maintained during the session.

Behaviour During the Interview: The patient was observed to engage in self-muttering during the interview and appeared internally preoccupied.

Psychomotor activity: Psychomotor activity was decreased, though goal-directed at times. Features of disinhibition and internal preoccupation were observed.

2. SPEECH:

Coherence: Speech was largely incoherent, with frequent derailment and illogical connections.

Incoherence: Present; speech was difficult to understand and fragmented.

Relevant to the question asked: Speech was not relevant and often deviated from the question asked.

Tone: Tone was audible.

Tempo: Speech was rapid, with pressure of speech observed.

Reaction time: Reaction time was increased, with responses often extending beyond the point and not appropriately timed.

Volume: Volume was adequate.

3. THOUGHT:

Form: There were disturbances in the form of thought, including loosening of associations, derailment, circumstantiality, and flight of ideas. The patient also demonstrated reduced capacity for abstract thinking in spontaneous speech.

Stream: The stream of thought was disorganized, with difficulty maintaining a single train of thought.

Possession: The patient reported experiences suggestive of thought interference, stating that thoughts were being inserted.

Content: Thought content was marked by bizarre and unrealistic beliefs, which were fragmented, fleeting, and frequently changing. There was noticeable preoccupation with religious, political, and philosophical themes.

4. MOOD AND AFFECT:

Subjective: The patient reported low mood, fatigability, and reduced emotional experience.

Objective: Affect appeared shallow, restricted, and incongruent with the content of speech and situation. Emotional expression appeared reduced and at times inappropriate, giving an overall empty quality.

5. SENSORIUM & PERCEPTION:

No evidence of illusions, hallucinations and pseudo-hallucinations were observed.

6. HIGHER COGNITIVE FUNCTIONS:

Level of Consciousness: The patient was conscious, alert, and responsive to stimuli.

Orientation: The patient was oriented to time, place, and person.

Attention and Concentration: Attention and concentration appeared superficially adequate; however, sustained attention was affected due to internal preoccupation.

Memory: The patient's memory functions were assessed across immediate, recent, and remote domains. Immediate memory was impaired, as the patient required increased time and effort to recall items and showed difficulty in retaining information briefly. Recent and remote memory appeared intact, as he was able to recall past and recent events adequately. The impairment in immediate memory appeared to be influenced by reduced attention and internal preoccupation.

Abstractability: Abstract thinking appeared largely intact on formal testing. The patient was able to identify similarities and differences and provide basic interpretations of proverbs; however, responses were often circumstantial and over-elaborated.

General intelligence and comprehension: The patient demonstrated adequate comprehension of questions and instructions, although responses were often circumstantial. Basic arithmetic abilities were intact, and overall intellectual functioning appeared to be within average range.

Judgement:

1. **Personal Judgment:** Impaired. The patient's responses reflected aimlessness and lack of purposeful planning (e.g., stating he would watch television and overthink).
2. **Future Judgment:** Impaired, with vague and circumstantial responses and absence of clear future planning.
3. **Social Judgment:** Impaired. Responses appeared empty and lacked practical relevance or goal direction.
4. **Test Judgment:** Responses were overly elaborated and circumstantial, indicating impaired practical reasoning.

Insight: Insight was assessed to be Grade 1 (Denial of illness), as the patient does not acknowledge having any psychological difficulties and lacks awareness of his condition.

TEST ADMINISTERED

1. Brief Psychiatric Rating Scale (BPRS)

The BPRS was administered to obtain a structured and objective assessment of the patient's current symptom severity. Given the presence of disorganized speech, unusual beliefs, emotional blunting, and behavioral disturbances, this scale helps in systematically quantifying the intensity of these symptoms. It allows for evaluation across multiple domains such as thought disturbance, mood, and behavior, and is particularly useful in establishing a baseline level of functioning.

2. Positive and Negative Syndrome Scale (PANSS)

The PANSS was used to assess the range and severity of the patient's symptoms in a more detailed manner, especially focusing on distinguishing between different types of symptoms such as unusual beliefs, reduced emotional expression, and social withdrawal. It provides a comprehensive profile of symptom patterns, which is essential for understanding the nature of the condition, planning management, and tracking progress over time.

3. Draw-A-Person Test (DAPT)

The DAPT was administered to gain insight into the patient's self-perception, emotional functioning, and underlying psychological conflicts through a non-verbal medium. Considering the patient's disorganized speech and difficulty in articulating thoughts, this test serves as an alternative way to understand his inner world. It can help reveal aspects of self-image, emotional distress, and possible disturbances in perception and thinking.

4. Millon Clinical Multiaxial Inventory-III (MCMI-III)

The MCMI-III was administered to assess the patient's underlying personality patterns and long-standing emotional and behavioral tendencies. Since the patient has a history suggestive of certain premorbid personality traits such as high ambition, sensitivity to stress, and social withdrawal, this test helps in understanding how these traits may have contributed to the development and maintenance of his current difficulties.

5. Sack's Sentence Completion Test (SSCT)

The SSCT was used as a projective technique to explore the patient's underlying thoughts, feelings, conflicts, and attitudes in a semi-structured manner. Given that the patient has difficulty expressing himself clearly and tends to give disorganized responses, this test allows for indirect expression of internal experiences.

6. Thematic Apperception Test (TAT)

The TAT was used to explore the patient's underlying needs, conflicts, emotional themes, and interpersonal perceptions through storytelling. Given the patient's tendency toward rambling and fragmented speech, this test allows for a more structured yet expressive way to understand recurring patterns in his thoughts. It helps in identifying themes related to relationships, fears, aspirations, and internal struggles, as well as the way he interprets and organizes experiences.

7. Rorschach Inkblot Test

The Rorschach test was administered to obtain an in-depth understanding of the patient's thought processes, emotional functioning, and reality perception. In this case, where there is marked disorganization in thinking, unusual beliefs, and difficulty maintaining coherent thought, the test helps assess how the patient perceives and interprets ambiguous stimuli. It is particularly useful in identifying disturbances in thought organization, emotional regulation, and contact with reality, as well as providing information about underlying personality structure and coping mechanisms.

RESULT AND INTERPRETATION:

The Brief Psychiatric Rating Scale (BPRS)

Scores Obtained	Level of psychiatric disturbance
56	Severe level of psychiatric disturbance

High scores on conceptual disorganization, unusual thought content, and grandiosity reflect significant disturbance in thinking, with disorganized, illogical, and unrealistic ideas. A blunted affect score along with emotional withdrawal suggests marked reduction in emotional expression and social disengagement. Elevated scores on hostility and uncooperativeness indicate irritability and occasional resistance. Lower scores on anxiety, suspiciousness, and somatic concern suggest these features are less prominent, while depressive mood appears mild.

The findings indicate severe impairment characterized by disorganized thinking, unrealistic beliefs, reduced emotional expression, and social withdrawal.

Positive and Negative Syndrome Scale (PANSS)

Scales	Scores
1. Positive Scale	30
2. Negative Scale	24
3. General psychopathology scale	39

The Positive and Negative Syndrome Scale (PANSS) profile indicates a marked level of psychopathology, with prominent disturbances across positive, negative, and general psychopathology scales. .

On the positive symptom domain, there is a strong presence of unrealistic and exaggerated beliefs, along with severe disturbance in the organization of thought, at times reaching a level where speech becomes difficult to comprehend. There is also increased rate of speech and mild irritability. Perceptual disturbances and suspiciousness appear to be minimal.

The negative symptom domain reflects significant reduction in emotional expression, decreased social engagement, and poor interpersonal connection. The individual shows clear withdrawal from social interaction and limited responsiveness to others. There is some difficulty in higher-level thinking tasks, although spontaneity and rigidity in thinking appear relatively preserved.

In the general psychopathology domain, the profile suggests mild low mood and guilt, along with some degree of uncooperativeness. There is a pronounced presence of unusual thought content and reduced awareness of difficulties. Motivation appears reduced, with noticeable lack of initiative and increased preoccupation with internal thoughts. Social avoidance is also evident.

The PANSS findings indicate a clinical picture characterized by severely disorganized thinking, prominent unrealistic beliefs, social withdrawal, reduced emotional expression, and impaired awareness, leading to significant disruption in overall functioning.

Draw-A-Person Test (DAPT)

The Draw-A-Person Test reveals significant disturbances in personality organization, self-concept, and emotional functioning. The overall drawings are characterized by fragmentation, poor integration, and lack of detail, indicating marked personality disorganization and impaired internal cohesion.

The male figure suggests compensatory grandiosity and inflated self-focus, likely masking underlying feelings of inadequacy, insecurity, and low self-worth. The presence of a disproportionately large head, combined with fragmented structure and fantasy elements, reflects preoccupation with internal thoughts, reliance on fantasy, and disturbed reality integration. Omission and distortion of facial features further point toward social withdrawal, reduced environmental engagement, and impaired interpersonal relatedness.

Both drawings demonstrate low energy, reduced drive, and depressive features, as reflected in light pressure, minimal detailing, and simplified forms. The repeated fragmentation and discontinuity in line quality indicate high internal conflict, anxiety, and breakdown in organized thinking processes.

There are also indicators of poor impulse control and emotional regulation, along with ambivalence toward independence and autonomy, as reflected in inconsistent structural elements such as feet orientation and posture. Features such as omission of hands and arms suggest feelings of helplessness, inadequacy, and lack of confidence in social and functional roles.

The drawings further reflect interpersonal difficulties, particularly themes of rejection, withdrawal, and difficulty forming secure relationships, possibly linked to early relational experiences. Elements suggesting dependency conflicts and unmet emotional needs are also evident. Indicators such as body segmentation and emphasis on certain body regions suggest underlying conflicts related to bodily concerns and internal tension, along with poor integration of physical and emotional self-experience. Findings are indicative of severe disturbances in thought organization, impaired reality testing, emotional withdrawal, internal preoccupation, and compromised self-structure, consistent with the clinical presentation of chronic psychological disturbance with disorganized and withdrawn functioning.

Feature Observed	Interpretation
Figure fills entire page	Compensatory self-expansiveness; underlying feelings of inadequacy
Light pressure	Hesitancy, insecurity, anxiety, possible depressive features
Vertical strokes	Underlying assertiveness and determination
Light, uncertain lines (chest emphasis)	Sensitivity to physical inadequacy
Large head	Inflated self-focus, fantasy involvement, possible grandiosity and egocentricity
Fragmented/robot-like figure	Poor impulse control; personality disorganization
Omitted facial features	Social withdrawal, poor interpersonal engagement
Hollow eyes	Reluctance to interact with environment
Immature nose (button/triangle)	Regression; emotional immaturity
Single-line unsmiling mouth	Depressive affect
Misplaced ears	Disturbance in perception; possible severe thought disturbance
Sparse/unshaded hair	Reduced vitality; lowered libido; depressive tendencies
Chin with light lines	Conflict between dominance needs and fear of responsibility

Short/slashed neck	Poor impulse control; possible self-harm ideation
Small hands	Feelings of helplessness and inadequacy
Poor hand structure	Immature emotional functioning
Large trunk	Unfulfilled drives and needs
Heavy separation line (lower body)	Sexual conflict
Long legs	Need for independence
Feet in opposite directions	Ambivalence toward autonomy
Dim/omitted hands and feet	Withdrawal; schizoid tendencies
Feet wide apart	Compensatory assertiveness; insecurity
Rigid posture	Emotional rigidity; constricted affect
Underclothed figure	Body preoccupation; introversion; fantasy orientation
Empty/poor detailing	Feelings of emptiness; withdrawal

Millon Clinical Multiaxial Inventory-III (MCMI-III)

- Modifying Indices

The response protocol was found to be valid, with no significant inconsistency or random responding detected. There was minimal likelihood that factors such as poor comprehension, attentional difficulties, fatigue, or carelessness significantly influenced the responses.

The Desirability Scale (Y) was low, indicating that the individual did not attempt to present himself in an overly favorable or socially desirable manner. In contrast, the Debasement Scale (Z) was elevated, suggesting a tendency to present oneself in a more negative or distressed light, possibly exaggerating psychological difficulties. This profile may reflect a “cry for help” pattern, where the individual emphasizes distress due to significant internal turmoil rather than deliberate distortion.

- Clinical personality patterns

Scale	BR Score	Clinical Significance
Schizoid	100	Pathological level
Avoidant	82	Presence of a syndrome
Depressive	87	Pathological level
Dependent	91	Pathological level
Masochistic	88	Pathological level

On the Schizoid scale (Scale 1), the profile suggests a tendency toward emotional detachment, social withdrawal, and limited capacity to experience pleasure. The individual appears indifferent to interpersonal relationships and functions in a distant, observer-like manner.

The Avoidant scale (Scale 2A) indicates heightened sensitivity to rejection and a tendency to withdraw from relationships due to fear of negative experiences. Despite a desire for connection, the individual maintains interpersonal distance as a protective strategy.

The Depressive scale (Scale 2B) reflects a pervasive pattern of pessimism, low mood, reduced pleasure, and a sense of hopelessness. There appears to be a chronic expectation of negative outcomes and diminished motivation.

A marked elevation on the Dependent scale (Scale 3) suggests strong reliance on others for emotional support, guidance, and decision-making. The individual demonstrates low autonomy, passivity, and a tendency to submit to others to maintain relationships.

The Masochistic/Self-Defeating scale (Scale 8B) indicates a pattern of self-sacrifice, self-criticism, and tolerance of unfavorable treatment. The individual may engage in behaviors that reinforce distress, accompanied by a tendency to focus on personal shortcomings and anticipate negative outcomes.

- Severe personality pathology

Scale	BR Score	Clinical Significance
Borderline	76	Presence of a syndrome

On the Borderline scale (Scale C), the score falls at a trait level elevation, suggesting underlying personality instability. This is characterized by fluctuations in mood, emotional dysregulation, and ambivalence in relationships. There may be difficulties in maintaining a stable sense of self, along with conflicting emotional experiences and heightened sensitivity to interpersonal dynamics.

- Clinical syndromes

Scale	BR Score	Clinical Significance
Anxiety	96	Pathological level

The Anxiety scale (Scale A) is elevated, indicating the presence of generalized tension and apprehension. The individual is likely to experience restlessness, hypervigilance, and difficulty relaxing, along with excessive worry and sensitivity to potential stressors. Physical manifestations of anxiety, such as bodily discomfort and autonomic arousal, may also be present.

- Severe clinical syndromes

Scale	BR Score	Clinical Significance
Major Depression	94	Pathological level

A significant elevation on the Major Depression scale (Scale CC) indicates a severe level of depressive symptomatology. This includes pervasive low mood, hopelessness, reduced energy, and impaired functioning. The individual may experience difficulties in concentration, feelings of worthlessness or guilt, and a bleak outlook toward the future. Disturbances in sleep and energy levels are also likely, contributing to overall functional impairment

- The Grossman Facet Scale

The Grossman facet scales are meant to help explain why certain personality scales are high on the Clinical Personality Patterns and Severe Personality Pathology scales. They show which exact parts of personality functioning (e.g., self-image, interpersonal relations) are contributing to those high scores. By carefully looking at the facet scale results, we can identify which personality traits stand out the most for him.

The MCMI–III profile is valid and reflects a response style characterized by low desirability and elevated debasement, suggesting a tendency to present oneself in a distressed and self-critical manner, possibly as a cry for help. The personality pattern is marked by significant elevations on schizoid, avoidant, depressive, dependent, and self-defeating traits, indicating a withdrawn, emotionally detached individual with underlying feelings of inadequacy, pessimism, and strong dependency needs, along with a tendency toward self-sacrifice and self-devaluation. At a deeper level, borderline personality features are present at a trait level, reflecting emotional instability, ambivalence in relationships, and fluctuating internal states. In terms of clinical syndromes, there is prominent anxiety, characterized by heightened tension, worry, and hypervigilance, along with a severe elevation of major depression, indicating pervasive low mood, hopelessness, reduced energy, and impaired functioning. Overall, the profile suggests a severely distressed individual with chronic emotional difficulties, social withdrawal, dependency needs, and impaired coping, contributing to significant dysfunction in personal and interpersonal domains.

Sacks Sentence Completion Test (SSCT)

Domain	Interpretation
Family Area	The responses suggest minimal conflict within the family environment, indicating generally stable familial relationships. However, there are mild underlying tensions specifically related to the father figure, reflecting some degree of unresolved emotional discomfort or authority-related conflict. Overall, the family is not perceived as a major source of distress, though subtle relational strain is present.
Sex Area	The responses in this domain are limited and guarded, indicating reluctance to disclose personal information. Despite this, there is no clear evidence of conflict or distress related to sexual concerns. The pattern suggests inhibition, discomfort, or avoidance in discussing this area, rather than the presence of overt issues.
Interpersonal Relationships	The patient demonstrates mild interpersonal difficulties, particularly in areas involving responsibility, trust, and interaction with others. There is evidence of avoidance of responsibility, discomfort in assertiveness, and preference for dependence on others. Additionally, a significant level of underlying fear and social apprehension is evident, indicating insecurity in peer interactions and a tendency to withdraw or avoid challenging situations. Overall, the interpersonal style reflects anxious avoidance, low confidence, and dependency needs.
Self-Concept	The patient's self-concept appears conflicted and inconsistent, with elements of both self-doubt and overcompensation. There are indications of guilt, regret, and negative evaluation of past actions, along with a tendency to externalize or rationalize difficulties. At the same time, there are expressions of perceived capability and desire for achievement, though these are not stable or well-integrated. The patient reflects uncertainty about identity, fluctuating self-worth, and limited clarity about future goals, along with underlying frustration and emotional instability. Overall, the self-concept is fragile, ambivalent, and poorly consolidated.

Thematic Apperception Test (TAT)

The analysis of the subject's TAT responses indicates that the individual was able to generate narratives; however, the stories were often simplistic, repetitive, and at times loosely organized, suggesting reduced cognitive complexity and limited elaboration of themes. The narratives showed adequate basic comprehension, but lacked depth, consistency, and resolution, reflecting impaired higher-order organization and reduced psychological integration.

Across the stories, the heroes were frequently passive, dependent, or constrained figures, often experiencing helplessness, restriction, or lack of control over life events. In several instances, the hero was not clearly differentiated from others, or multiple figures were identified as heroes, suggesting diffuse identity boundaries and weak individuation. There was a strong tendency to project physical or situational limitations (e.g., disability, helplessness, abandonment), indicating underlying feelings of inadequacy and reduced personal agency.

A dominant pattern across narratives was the presence of dependency needs (nSuccorance, nAffiliation), with repeated emphasis on seeking support, protection, and emotional closeness. These affiliative needs were often accompanied by fear of loss, abandonment, or separation, as seen in stories involving absent caregivers, broken families, or waiting for significant others. At the same time, there was minimal evidence of autonomous or achievement-oriented behavior, indicating a marked lack of initiative and reduced goal-directed functioning.

Interpersonal relationships were depicted in two contrasting ways: either as idealized and supportive (siblings, caregivers, partners) or as conflictual and distressing (family fights, abandonment). However, even in conflict situations, the subject consistently adopted a passive, non-confrontational stance, avoiding aggression and instead relying on withdrawal, resignation, or endurance. This suggests a coping style characterized by avoidance, emotional suppression, and low assertiveness.

The environmental press across stories was largely characterized by deprivation, lack of control, and dependency on external support systems, including authority figures, family members, or even religious entities. In several narratives, the resolution of conflict was either absent or externally determined, indicating limited problem-solving ability and poor ego mastery.

Emotionally, the narratives reflect underlying distress, insecurity, and vulnerability, though these are not always explicitly expressed. Instead, affect appears flattened, simplified, or indirectly conveyed, consistent with restricted emotional awareness and expression. Defenses such as denial, idealization, withdrawal, and reliance on fantasy or external support were prominent across stories.

Overall, the TAT profile reflects a personality organization marked by dependency, passivity, reduced autonomy, and impaired coping, with underlying themes of helplessness, loss, and unmet emotional needs. Ego functioning appears compromised, particularly in areas of initiative, problem-solving, and integration, while reality orientation is partially preserved but simplified. The findings are consistent with other assessments (Rorschach, MCMI, SSCT, and clinical interview), indicating a pattern of chronic psychological disturbance characterized by social withdrawal, emotional constriction, and impaired adaptive functioning.

Rorschach Inkblot test

Structural Summary:

R =	24	Sum Y=	0	Xu% =	0.208
L =	0,84	m =	3	FC : CF+C=	1:1
EB =	3:2.5	a:p =	8:4	GHR:PHR=	2:4
EA =	5.5	XA%=	0.45	Pure C=	1
EBper =	-	Ma:Mp=	1:2	Human content=	9
eb =	8:1	WDA%=	0.375	Constriction Ratio=	0:2.5
es =	9	2AB+Art+Ay=	7	Pure H=	6
D =	-1	X-%=	0.5	Afr =	0.33
Adj es =	8	Sum 6=	7	Isolation Index/ R=	0
Adj D =	0	Lv 2=	2	Blends: R=	3:24
FM =	5	Wsum6=	39	3r+(2)/R=	0.5
Sum C' =	0	S- =	1	W:D:Dd=	6:10:7
Sum T =	1	P =	1	Fr+rF=	2
Sum V=	0	X+%=	0.25	W:M =	6:3
An+ Xy=	1	Zd=	+9.5	H: (H)+Hd+(Hd)=	6:3
Zf=	8	Zsum=	33.5	Zest=	24.0
PTI=	5	DEPI=	5	CDI=	3
S-CON=	7	HVI=	3	OBS=	2

Across domains, the protocol reflects a severely disorganized personality structure with marked impairment in reality testing, thought processes, emotional regulation, and interpersonal functioning. Despite superficially adequate control, the individual demonstrates fragile coping, high vulnerability to stress, and pervasive cognitive and perceptual distortions, consistent with a chronic psychotic condition with deteriorating functioning.

I. Control and stress tolerance

The protocol indicates apparently adequate but fragile control over stress, as reflected by an Adjusted D of 0, suggesting average coping capacity under routine conditions. However, this is contradicted by relatively low EA (5.5) and elevated es (9), indicating that the individual's available coping resources are limited relative to experienced demands. The presence of a negative D score (-1) further highlights vulnerability to situational overload, suggesting that control is not consistently maintained and may deteriorate under stress.

This fragility is reinforced by findings across domains. The over-incorporative processing style ($Z_d = +9.5$) and elevated cognitive load ($W_{sum6} = 39$) suggest excessive effort being invested in managing internal and external demands, often inefficiently. This aligns with MSE findings of disorganized speech, impaired judgment, and internal preoccupation, as well as BPRS/PANSS indicators of severe disorganization and impaired functioning. The discrepancy between perceived control and actual coping reflects a pseudo-adaptive pattern, where the individual may appear controlled but is internally overwhelmed.

Clinically, the individual is likely to function adequately in structured, low-demand environments, but shows marked vulnerability to disorganization, impulsivity, and cognitive breakdown in complex or ambiguous situations. This pattern is consistent with the chronic deterioration described in history and supports the presence of impaired adaptive regulation despite superficial control.

II. Situational Stress

The difference between D and Adjusted D, along with elevated es, indicates the presence of ongoing situational stress, which is significantly impacting functioning. The stress appears to interfere with attention, concentration, and cognitive clarity, as reflected in the ideation domain and supported by MSE observations of derailment, circumstantiality, and difficulty sustaining thought processes.

The absence of significant Y responses suggests that distress is not experienced as overt anxiety, but rather manifests as cognitive disorganization and internal strain, which is consistent with psychotic-level functioning. The elevated S-CON (7) further indicates risk for poor stress tolerance and maladaptive coping, including vulnerability to impulsive or poorly regulated behaviors.

This stress pattern is corroborated by case history factors, including chronic illness course, unemployment, social withdrawal, and financial stressors, as well as MCMI findings of high anxiety and major depression. The stress appears both situational and internally generated, contributing to a cycle where increasing cognitive and emotional demands further impair functioning.

III. Affect

The affective profile reflects marked emotional constriction and dysregulation. The low Afr (0.33) indicates avoidance of emotional stimuli and discomfort with affective experiences, while the FC:CF+C ratio (1:1) suggests inconsistent modulation of emotional expression. This implies that emotions are generally inhibited, but when expressed, may be poorly controlled or disproportionate.

The presence of DEPI = 5 highlights a vulnerability to depressive experiences, including low mood, internal distress, and emotional withdrawal. The absence of shading responses (Sum Y = 0, Sum V = 0) suggests that emotional distress is not consciously processed, but rather internalized or expressed indirectly through cognitive disturbance. This aligns with MCMI findings of severe depression and anxiety, as well as SSCT indicators of guilt, ambivalence, and unstable self-concept.

Additionally, reliance on intellectualization (supported by elevated cognitive activity and MCMI profile) suggests that emotions are managed defensively through cognitive processes. However, under increased stress, this defense becomes ineffective, leading to emotional disorganization, as seen in incongruent affect in MSE and shallow emotional expression. Overall, affect is restricted, inconsistently regulated, and contributes to internal instability.

IV. Information Processing

The processing style is characterized by inefficiency and inconsistency, with evidence of both over-involvement ($Z_d = +9.5$) and reduced effective scanning ($Z_f = 8$). This indicates that although the individual invests excessive effort in processing, the output remains poorly organized and ineffective.

The elevated number of Dd responses (7) suggests a tendency to focus on unusual or irrelevant details, contributing to disorganized perception. Additionally, the imbalance in W:M ratio (6:3) indicates that the individual may attempt to process more information than can be effectively managed, leading to frustration and cognitive overload.

These findings align with MSE observations of disorganized, rambling speech and difficulty maintaining coherence, as well as PANSS indicators of conceptual disorganization. The

processing inefficiency contributes directly to impaired functioning, particularly in complex environments, reinforcing the pattern of cognitive disorganization under demand.

V. Cognitive Mediation

There is severe impairment in reality testing, as indicated by low XA% (0.45), WDA% (0.375), and X+% (0.25), along with a high X-% (0.5). These findings suggest that the individual has significant difficulty accurately perceiving and interpreting reality, leading to distorted and unconventional interpretations.

The presence of PTI = 5 strongly supports the presence of psychotic-level disturbances, with pervasive impairment across contexts. The small difference between XA% and WDA% indicates that the dysfunction is global, affecting both simple and complex situations. This is consistent with clinical observations of bizarre beliefs and impaired insight, as well as BPRS/PANSS findings of unusual thought content and severe disorganization.

The low number of popular responses ($P = 1$) further indicates departure from shared reality and social conventions, suggesting that the individual's perceptions are highly idiosyncratic. Overall, mediation is markedly impaired, resulting in distorted perception, poor judgment, and behavior that may appear inappropriate or disconnected from reality.

VI. Ideation

The ideational profile reflects disorganized and disturbed thinking processes, with evidence of cognitive slippage and impaired conceptualization. The extremely elevated Wsum6 (39) indicates significant disturbance in thought processes, including illogical reasoning, inconsistency, and bizarre ideation.

The introversive style ($EB = 3:2.5$) suggests reliance on internal processing; however, this is compromised by distorted cognitive activity, leading to poor judgment and impaired decision-making. The imbalance in Ma:Mp (1:2) further indicates a tendency toward passive ideation and reduced active problem-solving, contributing to ineffective coping.

These findings are strongly supported by MSE observations of derailment, flight of ideas, and incomprehensible speech, as well as PANSS conceptual disorganization and grandiosity. The ideational disturbances also align with DAPT findings of fragmentation and fantasy orientation, indicating reliance on internal, often unrealistic cognitive processes. Overall, thinking is severely disorganized, poorly controlled, and disconnected from reality.

VII. Self perception

The self-perception is characterized by inflated yet unstable self-evaluation, as indicated by $Fr+rF > 0$ and elevated egocentricity index (0.5). This suggests narcissistic features, with a tendency toward exaggerated self-focus and need for self-validation.

However, this self-enhancement is not stable and coexists with underlying vulnerability and self-doubt, as reflected in SSCT responses showing guilt, regret, and ambivalence, as well as MCMI depressive and self-defeating traits. The self-image appears to be poorly integrated, fluctuating between inflated and diminished self-worth.

The presence of adequate human content (9, with Pure H = 6) suggests that self-concept is influenced by interpersonal experiences; however, given the distortion in perception and thinking, these representations are likely inaccurate or poorly integrated. Overall, the self-concept is fragile, conflicted, and inconsistently regulated, contributing to maladaptive functioning.

VIII. Interpersonal perception and behaviour

The interpersonal profile indicates significant difficulties in relationships, characterized by dependency, poor understanding of others, and maladaptive interaction patterns. The imbalance in GHR:PHR (2:4) suggests that interpersonal behaviors are often ineffective or inappropriate, leading to strained relationships.

Although there is some interest in relationships (Human content elevated, Sum T = 1), this is accompanied by limited social understanding and unrealistic expectations, which may result in social withdrawal or conflict. This pattern is consistent with SSCT findings of interpersonal fear and avoidance, as well as MCMI dependent and avoidant traits.

The interpersonal difficulties are further compounded by distorted perception (mediation deficits) and emotional constriction, leading to reduced capacity for meaningful engagement. Clinically, this manifests as social withdrawal, poor communication, and impaired relational functioning, as observed in the case history and MSE.

DIAGNOSTIC FORMULATION:

The patient is a 50-year-old unmarried male, educated up to BA LL.B., previously employed as a lawyer and currently unemployed, belonging to a middle socioeconomic background, residing with his father and brother. He was brought for assessment under coercion with limited awareness of his condition. He presents with an eight-year history of progressively worsening disturbances characterized by bizarre and grandiose delusions, including identity-related beliefs and impossible transformations, along with marked disorganization of thought and speech,

reflected in incoherent, illogical, and irrelevant verbal output with loosening of associations and derailment. These symptoms are accompanied by social withdrawal, self-muttering behavior, reduced goal-directed activity, poor self-care, wandering behavior, and significant occupational dysfunction, along with inappropriate, shallow, and incongruent affect. The onset of symptoms has been insidious, with a chronic and deteriorating course, and is associated with poor insight and minimal help-seeking behavior.

On mental status examination, the patient appeared adequately groomed but displayed reduced and poorly sustained eye contact, with observable self-muttering and preoccupation. His speech was pressured, disorganized, and difficult to comprehend, often not relevant to questions asked. Thought processes were loosely connected, fragmented, and difficult to follow, with evidence of disturbed flow of thought, circumstantiality, and flight of ideas, along with bizarre and shifting thought content. Affect was noted to be shallow, restricted, and incongruent with the content of speech, while mood appeared subjectively low and fatigued. Although orientation and basic cognitive functions were relatively intact, judgment was impaired and insight was at Grade I (denial of illness), indicating poor awareness of his condition and its consequences.

Findings from psychological assessment converge with the clinical picture and further substantiate the presence of a severe thought disorder. Rating scales indicate significant psychopathology across both positive and negative domains, with prominent conceptual disorganization, unusual thought content, emotional withdrawal, and reduced emotional expression. Personality assessment suggests a background of social detachment, dependency, pessimism, and low self-worth, along with vulnerability to emotional instability, which likely predisposed the individual to maladaptive coping under stress. Projective assessments reveal impairment in reality testing, distorted perception, and disorganized ideation, along with themes of helplessness, dependency, loss, and reduced personal agency. Emotional functioning appears constricted and poorly regulated, with reliance on withdrawal, avoidance, and internalization rather than active problem-solving, while interpersonal representations indicate dependency needs, fear of abandonment, and difficulty forming effective social relationships.

The overall pattern across clinical interview, behavioral observations, and psychological testing reflects a severely compromised personality organization, with disturbances in cognition, affect, and behavior that are pervasive and persistent. There is clear evidence of disorganized thinking, impaired reality testing, affective incongruity, and significant functional decline, which are consistent across multiple domains of assessment. The patient demonstrates fragile stress tolerance, reduced capacity for adaptive functioning, and marked impairment in social and occupational roles, with a tendency toward withdrawal and passivity in response to internal and external demands. The case reflects a chronic psychotic disorder with prominent disorganization and affective disturbance, accompanied by poor adaptive functioning and limited insight.

PROVISIONAL DIAGNOSIS:

F20.1 Hebephrenic Schizophrenia

POINTS IN FAVOUR:

1. Prominent disorganization of thought and speech: persistent incoherence, derailment, illogical and irrelevant responses, and inability to maintain a goal-directed train of thought.
2. Inappropriate and shallow affect: marked incongruity between emotional expression and content, with restricted affect.
3. Fragmented, rapidly shifting bizarre ideas: unstable, poorly systematized delusional content rather than fixed, organized beliefs.
4. Behavioral disorganization: aimless wandering, self-muttering, reduced self-care, and lack of purposeful activity.
5. Negative symptoms: social withdrawal, reduced initiative (avolition), emotional blunting, and diminished engagement with surroundings.
6. Chronic course with deterioration: insidious onset over ~8 years with progressive decline in social and occupational functioning.
7. Poor insight: denial of illness and lack of help-seeking behavior.
8. Supportive psychological findings: projective tests and personality assessment indicate impaired reality testing, disorganized ideation, dependency, and emotional constriction, consistent with a disorganized psychotic process.

DIFFERENTIAL DIAGNOSIS:

F06.2 Organic Psychotic Disorder

F22 Persistent Delusional Disorder

RECOMMENDATIONS AND SUGGESTIONS

1. Individual Psychotherapy: Long-term psychotherapy is strongly recommended, with a focus on anxiety management, emotional regulation, and cognitive restructuring. A structured approach such as Cognitive Behaviour Therapy (CBT) is indicated to address maladaptive thoughts related to health fears, self-blame, and negative future expectations.

Techniques for grounding and distress tolerance should also be incorporated to manage dissociative episodes.

2. **Family Intervention:** Intervention should focus on improving parent–child communication, emotional responsiveness, and supportive caregiving practices, while reducing criticism and conflict within the home environment.
3. **Psychoeducation:** Psychoeducation should be provided to the patient and caregivers regarding the nature of anxiety, depressive, and dissociative symptoms and role of stress and emotional factors in physical symptoms. This would help in reducing misconceptions, blame, and health-related anxiety.
4. **Follow-Up and Monitoring:** Regular follow-up sessions are essential to monitor symptom progression, assess risk related to suicidal ideation and modify intervention strategies as required

